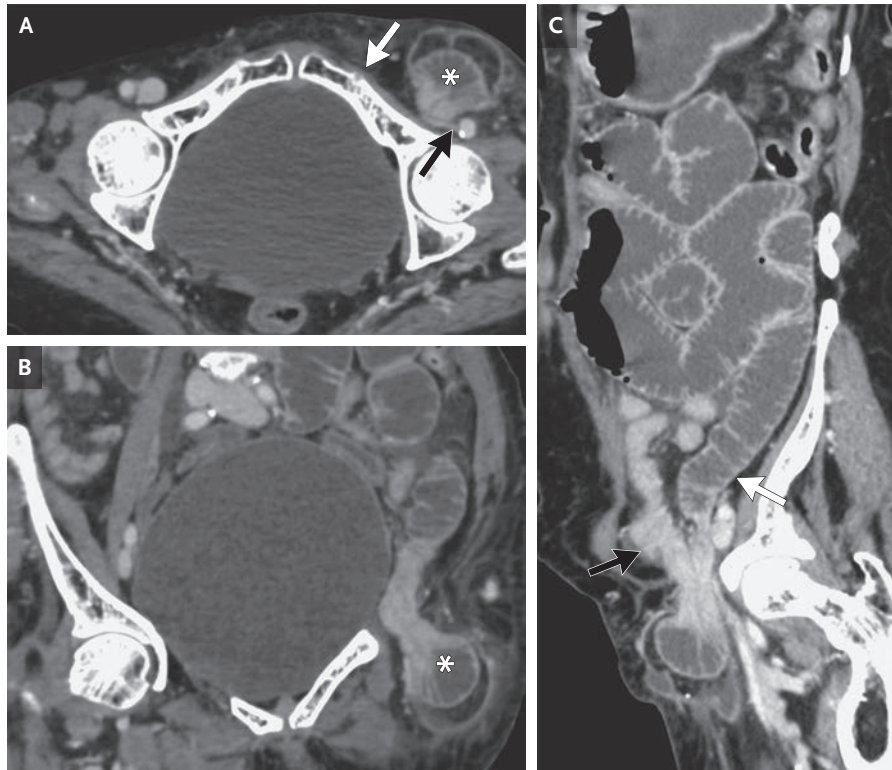


IMAGES IN CLINICAL MEDICINE

Incarcerated Femoral Hernia



A 77-YEAR-OLD WOMAN PRESENTED TO THE EMERGENCY DEPARTMENT with a 2-day history of abdominal pain, nausea, and vomiting. On physical examination, abdominal distention with tenderness was present in the left lower quadrant. Computed tomography of the abdomen revealed herniation of a loop of ileum on the left side (Panels A and B, asterisks) that was inferior to the inguinal ligament, lateral to the pubic tubercle (Panel A, white arrow), and medial to the common femoral vessels, with compression of the femoral vein (Panel A, black arrow). Small bowel loops were dilated proximal to the herniation (Panel C, white arrow) and were collapsed distal to the herniation (Panel C, black arrow). The bladder was distended because the patient had not voided before the study. A diagnosis of incarcerated femoral hernia was made. Femoral and inguinal hernias are distinguished by their protrusion below and above the inguinal ligament, respectively. Femoral hernias are much less common than inguinal hernias and occur more frequently in women than in men. Owing to the small size of the femoral ring, femoral hernias are more likely to be associated with incarceration and strangulation than inguinal hernias. Therefore, surgical repair of femoral hernias is recommended before complications develop. In this patient, an emergency laparotomy with repair of the hernia was performed. Postoperatively, she recovered well.

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